SACTCHECK · REGIMEN-SPECIFIC SACT CONSENT

PROTOTYPE · GOVERNANCE REVIEW REQUIRED

Pembrolizumab every 3 weeks for head and neck cancer

First-line treatment of metastatic or unresectable recurrent head and neck squamous cell carcinoma with a tumour PD-L1 combined positive score (CPS) of at least 1. This indication excludes nasopharyngeal cancer and cancer progressing within six months after curatively intended systemic treatment for locoregionally advanced disease. · NCCP 00455 · version 16 · 21-day cycle

Patient name	DOB	Hospital no.	Date

Treatment context
Pembrolizumab aims to control head and neck cancer that has returned or spread, relieve symptoms and help some people live longer. Benefit and side effects are reviewed for you.

Schedule
Pembrolizumab 200 mg is given through a vein over about 30 minutes on Day 1 every 3 weeks. The whole visit includes safety checks, preparation and observation. Treatment continues until progression or unacceptable toxicity. After 24 months of sustained response without progression, the team may consider stopping; stopping at 24 months is not automatic.

1. TREATMENT DISCUSSION

- ☐ Purpose, expected benefit and alternatives explained
- ☐ Indication, medicines, sequence and duration confirmed
- ☐ Blood tests and any individually indicated heart checks discussed
- ☐ Support medicines and phase changes explained
- ☐ Fertility, contraception and breastfeeding discussed where relevant
- ☐ Common, serious and lasting drug-specific risks discussed below
- ☐ Urgent symptoms and local contact details confirmed

2. REGIMEN / AGENT-SPECIFIC MATERIAL RISKS

Pembrolizumab
IMMUNOTHERAPY

- ☐ Immune inflammation of lungs, bowel, liver, kidneys, hormone glands, skin, heart and nervous system
- ☐ Infusion reactions
- ☐ Some effects need steroids, hospital care or long-term hormone replacement
- ☐ Effects may start after the final dose
- ☐ Heart, muscle and nerve immune effects can occur together and require urgent assessment

Discuss your diagnosis, intent, alternatives, expected benefit and uncertainty, treatment burden, serious or lasting harms, fertility and pregnancy precautions and your priorities. Include immune inflammation of lungs, bowel, liver, kidneys, skin, glands, heart, muscles, nerves and eyes; infusion or injection reactions; delayed effects; possible high-dose steroid treatment, permanent hormone replacement or permanent treatment stopping. Confirm actual urgent contacts and use teach-back. This draft does not authorise treatment.

Individual aim / benefit, alternatives, material risks and questions:

3. CONSENT CONFIRMATION

- ☐ Expected benefit, important risks and reasonable alternatives discussed
- ☐ Patient questions invited and answered; patient-specific material risks / priorities considered
- ☐ Emergency contact / toxicity advice and written information offered where appropriate
- ☐ Patient understands consent is voluntary and may be withdrawn

Patient / person giving consent _____ Date _____

Clinician obtaining consent _____ Date _____

Prototype only. This one-page form is a regimen-specific consent aid and requires formal governance, consultant and oncology-pharmacy review before clinical use. The current NCCP regimen, local/national consent policy, current medicine information, patient-specific material risks and clinical judgement remain authoritative.
Base regimen: NCCP 00455 v16.

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